INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Nivolumab 3 mg/kg + ipilimumab 1 mg/kg — advanced melanoma

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
2
Explained in the treatment reference

Regimen source
NCCP 00551
Version 7a

Reference treatment cycle

Treatment stages

01 **Combination induction**
Up to four doses

02 **Nivolumab alone**
Ongoing review

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Immunotherapy can inflame healthy organs during treatment or after it stops. Contact the oncology team promptly for new or worsening diarrhoea, blood or mucus in stools, abdominal pain, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell every treating clinician that you are receiving or have received immunotherapy.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

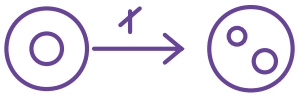
What each medicine does

To help immune cells control advanced melanoma with two checkpoint medicines, followed when appropriate by nivolumab alone. The chosen dose combination has specific licensing and evidence limitations to discuss during consent.

PD-1 CHECKPOINT IMMUNOTHERAPY

Nivolumab

Opdivo



Releases a PD-1 immune brake

Nivolumab blocks PD-1, helping T cells remain active against melanoma. With ipilimumab it acts on two immune checkpoints. This can also cause healthy-organ inflammation; changing to nivolumab alone does not erase earlier immune side effects.

Releases a PD-1 immune brake → Treatment plan and safety checks → Report new symptoms early

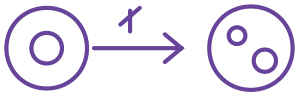
What to watch for

Carry the immunotherapy alert card and report new symptoms during treatment and after it stops.

CTLA-4 CHECKPOINT IMMUNOTHERAPY

Ipilimumab

Yervoy



Releases a CTLA-4 immune brake

Ipilimumab blocks CTLA-4, one of the signals that normally restrain T cells. This may help immune cells attack melanoma. The same immune activity can inflame healthy organs, sometimes after the last infusion; some hormone problems need lasting treatment.

Releases a CTLA-4 immune brake → Treatment plan and safety checks → Report new symptoms early

What to watch for

Report diarrhoea, blood in stools, severe headache, unusual weakness or spreading rash promptly.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Understanding your diagnosis and treatment goal

Use your own pathology, stage and agreed plan alongside this guide.

Your diagnosis and stage

Record where the melanoma began, its pathology result and the sites involved now. This pathway treats melanoma that cannot be fully removed or has spread. Treatment after complete removal has a different aim and a separate plan.

The tumour test

The laboratory checks a sample of the melanoma for a BRAF V600 change. The result can identify a target for certain medicines. A change found in the tumour does not, by itself, establish an inherited condition.

Scans and local treatments

Add planned scans, any operation and any radiotherapy to the same calendar. Ask what each part is intended to achieve and which team arranges it.

New changes still matter

Tell your team about a changing mole, a persistent sore, a new lump or an unexplained symptom. Improvement in a known tumour does not explain a new problem elsewhere.

Your medicines and overall plan

The schedule

Up to four combination infusions, 21 days apart, then nivolumab alone every two or four weeks as prescribed; four stamps record combination doses only.

How treatment works

Nivolumab blocks PD-1, helping T cells remain active against melanoma. With ipilimumab it acts on two immune checkpoints. This can also cause healthy-organ inflammation; changing to nivolumab alone does not erase earlier immune side effects. Ipilimumab blocks CTLA-4, one of the signals that normally restrain T cells. This may help immune cells attack melanoma. The same immune activity can inflame healthy organs, sometimes after the last infusion; some hormone problems need lasting treatment.

Important boundaries

NCCP identifies nivolumab 3 mg/kg plus ipilimumab 1 mg/kg for melanoma as an unlicensed dosing posology in Ireland, despite reimbursement. Your clinician should explain this, the available alternatives and uncertainties, document informed consent under hospital policy, and follow the hospital Drug and Therapeutics Committee or equivalent acknowledgement process. CheckMate 511 primarily compared safety; it did not prove that this schedule has equal survival benefit to nivolumab 1 mg/kg plus ipilimumab 3 mg/kg.

Evidence and choices

Grade 3–5 treatment-related adverse events occurred in approximately 34% versus 48% (p=0.006). These are group results from that study, not a prediction for you. The study was not designed to establish efficacy noninferiority or equivalent survival. Do not assign CheckMate 067 long-term survival estimates for the other dose combination to this schedule. NCCP states the melanoma 3/1 dosing posology is unlicensed in Ireland; local consent and governance requirements apply.

Decisions, support and useful records

Your priorities belong in the treatment discussion.

Benefits and alternatives

Ask what the treatment aims to achieve, what alternatives are suitable, how likely important benefits are in your situation and how treatment burden or uncertainty affects that choice. A trial average cannot promise an individual outcome.

Support alongside treatment

Supportive care addresses symptoms and daily life during cancer treatment. Palliative care can be part of that support; accepting it does not mean you must stop anticancer treatment.

A person you trust

Choose whether to involve a relative or supporter and what may be shared with them. Ask for an interpreter or a different information format. Check that you can explain the next dose, tests and urgent-contact plan in your own words.

Your document set

Keep the current treatment summary, medicine list, test plan and contact card together. Use the printed instructions when you need them; the QR code is an optional link to further information.

What the treatment evidence can and cannot tell you

Evidence needs to match the medicine, dose combination, disease stage and previous treatment.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Your treatment phases

Treatment stages

01 **Combination induction**
Up to four doses

02 **Nivolumab alone**
Ongoing review

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

UP TO FOUR DOSES

Combination induction

Nivolumab 3 mg/kg then ipilimumab 1 mg/kg | **Every 21 days**

NCCP identifies nivolumab 3 mg/kg plus ipilimumab 1 mg/kg for melanoma as an unlicensed dosing posology in Ireland, despite reimbursement. Your clinician should explain this, the available alternatives and uncertainties, document informed consent under hospital policy, and follow the hospital Drug and Therapeutics Committee or equivalent acknowledgement process. CheckMate 511 primarily compared safety; it did not prove that this schedule has equal survival benefit to nivolumab 1 mg/kg plus ipilimumab 3 mg/kg.

ONGOING REVIEW

Nivolumab alone

Every two or four weeks as prescribed | **intravenous (IV) or subcutaneous route selected by the team**

If a dose is delayed or held, obtain a revised written plan. The first two-weekly maintenance dose starts three weeks after the last combination dose; the first four-weekly dose starts six weeks afterwards. Do not calculate a different start from the maintenance interval alone. Earlier immune toxicity and permanent-stop decisions still apply.

BASELINE → TREATMENT → FOLLOW-UP

Your testing and review plan

A reference cycle is not a personalised calendar. Tests listed as symptom-triggered or individual are not presented as mandatory tests at every visit. No universal imaging or tumour-marker interval is invented.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00551 v7a: current regimen source

THE WHOLE TREATMENT JOURNEY

Nivolumab 3 mg/kg + ipilimumab 1 mg/kg — advanced melanoma: treatment and checks

Advanced melanoma

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Safety tests and symptoms NCCP 00551	Blood count, kidney and liver function, glucose, thyroid tests and BRAF tumour status. Review autoimmune disease, previous immune reactions, transplantation, infection and all medicines. Confirm informed consent for this unlicensed melanoma dosing schedule and the local governance requirements.	Blood count, kidney/liver tests, glucose and thyroid tests before every cycle, with symptom review during combination and maintenance phases. Cortisol and other organ tests are added when indicated. Recheck route, dose, interval and the maintenance start date when phases change.	Immunotherapy can inflame healthy organs during treatment or after it stops. Contact the oncology team promptly for new or worsening diarrhoea, blood or mucus in stools, abdominal pain, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell every treating clinician that you are receiving or have received immunotherapy.	NCCP specifies ongoing monitoring for at least five months after the last dose. Immune effects can require longer follow-up, steroid tapering or lasting hormone replacement. Response scans and any further melanoma treatment follow the individual plan.	Do not restart held treatment until the team confirms the medicine, dose and timing. Safety tests and unresolved symptoms are reviewed first.
Scans and tumour results NCCP 00551	Keep the pathology, stage and relevant biomarker report.	Response scans follow the individual disease plan, separate from safety bloods.	Report new symptoms rather than waiting for a scan.	Ask which scan, its planned date and who will explain the result.	A treatment hold or stop may change the next scan or follow-up date.

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Written plan and contacts NCCP 00551	Record actual oncology and out-of-hours numbers; examples are not working contact details.	Bring your medicine list and record the dose actually given or taken.	Keep the wallet card and written urgent-symptom plan available.	Ask for a printed updated plan when treatment changes.	Keep the last-dose date and any steroid, hormone or other follow-up instructions.

What each check is for

1 **Combination induction**

Up to four doses

To help immune cells control advanced melanoma with two checkpoint medicines, followed when appropriate by nivolumab alone. The chosen dose combination has specific licensing and evidence limitations to discuss during consent.

Ask / act: Confirm the current phase, prescription, tests and support medicines.

2 **Nivolumab alone**

Ongoing review

To help immune cells control advanced melanoma with two checkpoint medicines, followed when appropriate by nivolumab alone. The chosen dose combination has specific licensing and evidence limitations to discuss during consent.

Ask / act: Confirm the current phase, prescription, tests and support medicines.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Taking or receiving treatment

During combination treatment, nivolumab 3 mg/kg is infused first over 30 minutes, then ipilimumab 1 mg/kg over 30 minutes. Maintenance may be nivolumab IV 240 mg every two weeks or IV 480 mg every four weeks, or the prescribed subcutaneous alternative of 600 mg every two weeks or 1200 mg every four weeks. In this melanoma source, IV 240 mg takes 30 minutes and IV 480 mg takes 60 minutes; subcutaneous administration takes about three to five minutes.

STEP 2

Missed, vomited or held treatment

If a dose is delayed or held, obtain a revised written plan. The first two-weekly maintenance dose starts three weeks after the last combination dose; the first four-weekly dose starts six weeks afterwards. Do not calculate a different start from the maintenance interval alone. Earlier immune toxicity and permanent-stop decisions still apply.

STEP 3

Appointments and supply

Before each visit, check four things: the blood-test booking, how you will get there, your medicine supply and any help you need. Ask who will review the results and tell you whether the next dose can go ahead.

STEP 4

Paper copies and contacts

Keep the handbook, short guide and treatment card in one place. Ask your team to fill in the real daytime and emergency numbers and print a replacement when the plan changes. A phone is optional.

Taking or receiving treatment safely

Keep the written prescription and actual treatment record together.

Other medicines

Give the team a complete list of prescribed, over-the-counter and herbal medicines, supplements and allergies. Ask before adding a new medicine; treatment interactions and symptom-masking can matter.

Practical arrangements

Write down where blood samples are taken, who receives the results and how you obtain the next appointment or medicine supply. Request a printed plan if that is easier to use.

After a treatment hold, change or completion

The next step should be written down, not assumed.

Follow-up and late effects

NCCP specifies ongoing monitoring for at least five months after the last dose. Immune effects can require longer follow-up, steroid tapering or lasting hormone replacement. Response scans and any further melanoma treatment follow the individual plan.

A treatment summary

Ask for a treatment summary showing what you received, important side effects, admissions, the last dose and the follow-up team. Share the current version with your GP and other clinicians.

Your next review

Before leaving, confirm the next test and review, how you will receive results and whom to call sooner. Keep those numbers available after treatment ends.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Treatment visit	 **Day 1** During combination treatment, nivolumab 3 mg/kg is infused first over 30 minutes, then ipilimumab 1 mg/kg over 30 minutes. Maintenance may be nivolumab IV 240 mg every two weeks or IV 480 mg every four weeks, or the prescribed subcutaneous alternative of 600 mg every two weeks or 1200 mg every four weeks. In this melanoma source, IV 240 mg takes 30 minutes and IV 480 mg takes 60 minutes; subcutaneous administration takes about three to five minutes.
Between-visit monitoring	 **Days 2–21** Immunotherapy can inflame healthy organs during treatment or after it stops. Contact the oncology team promptly for new or worsening diarrhoea, blood or mucus in stools, abdominal pain, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell every treating clinician that you are receiving or have received immunotherapy.

Schedule and review: Up to four combination infusions, 21 days apart, then nivolumab alone every two or four weeks as prescribed; four stamps record combination doses only.

Holds, missed treatment and phase changes: If a dose is delayed or held, obtain a revised written plan. The first two-weekly maintenance dose starts three weeks after the last combination dose; the first four-weekly dose starts six weeks afterwards. Do not calculate a different start from the maintenance interval alone. Earlier immune toxicity and permanent-stop decisions still apply.

> **Why these bands stay broad:** This is a reference schedule. Your prescribed dose, blood results, symptoms and the team's written plan determine whether treatment goes ahead. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00551 v7a: current regimen source ↗

There are no guaranteed "safe days". The prescribed dose, treatment decision and local urgent-contact plan take priority. Call 112 or 999 for severe breathing difficulty, severe chest pain, collapse, a new seizure, sudden weakness or speech difficulty, or uncontrolled bleeding. Follow your oncology unit's written urgent-contact plan for other new or worsening symptoms; do not wait for the next appointment.

Between-treatment notes

Day 1: Treatment day

During combination treatment, nivolumab 3 mg/kg is infused first over 30 minutes, then ipilimumab 1 mg/kg over 30 minutes. Maintenance may be nivolumab IV 240 mg every two weeks or IV 480 mg every four weeks, or the prescribed subcutaneous alternative of 600 mg every two weeks or 1200 mg every four weeks. In this melanoma source, IV 240 mg takes 30 minutes and IV 480 mg takes 60 minutes; subcutaneous administration takes about three to five minutes. Report symptoms and previous reactions before treatment.

Day 2–21: Between treatment visits

Immunotherapy can inflame healthy organs during treatment or after it stops. Contact the oncology team promptly for new or worsening diarrhoea, blood or mucus in stools, abdominal pain, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell every treating clinician that you are receiving or have received immunotherapy. Do not wait for the next appointment to report urgent symptoms.

Support around the medicines

1 Supportive medicines and symptom plan

NCCP classifies the combination phase as low emetogenic risk and nivolumab alone as minimal risk. Anti-sickness treatment is individual. Routine infusion premedication is not usually required; a previous reaction may alter that plan. Steroid or hormone treatment for immune effects must have a written follow-up plan.

2 Skin and daily care

Use gentle skin care and sun protection. Report rash early and obtain urgent assessment for spreading painful rash, blisters, peeling or mouth/eye/genital sores. Continue melanoma self-checks and planned skin reviews during maintenance and after treatment.

3 Fertility, pregnancy and breastfeeding

Use effective contraception during treatment and for at least five months after the final nivolumab dose. Discuss fertility and breastfeeding before starting and tell the team promptly if pregnancy is possible. Keep the last-dose date on the treatment record.

Tests, results and treatment decisions

Safety tests and scans answer different questions.

Before treatment

Blood count, kidney and liver function, glucose, thyroid tests and BRAF tumour status. Review autoimmune disease, previous immune reactions, transplantation, infection and all medicines. Confirm informed consent for this unlicensed melanoma dosing schedule and the local governance requirements.

During treatment

Blood count, kidney/liver tests, glucose and thyroid tests before every cycle, with symptom review during combination and maintenance phases. Cortisol and other organ tests are added when indicated. Recheck route, dose, interval and the maintenance start date when phases change.

What a result means

Ask which result is abnormal, whether the finding needs a repeat test and whether treatment is held, changed or stopped. Do not use a reference range or one normal result to decide independently that a dose is safe.

What to expect between visits

A quiet treatment day does not rule out later problems.

Supportive medicines

A useful daily record

Record symptoms, temperature when unwell, bowel changes, fluid intake when reduced, missed doses and any medicines used for relief. Note how symptoms affect eating, walking, sleep and normal activities; this helps the team judge severity.

When life is affected

Bring up difficulties with eating, sleep, pain, energy, work or caring for someone. The team can arrange symptom support while cancer treatment continues.

Skin, body changes and daily care

Use the detailed body map to locate symptoms, not to diagnose them.

The body map

The organ anchors show possible locations and warning symptoms. They do not show how common each effect is, and a symptom may have another cause. New symptoms should be assessed in their clinical context.

Food, fluids and activity

Choose food, fluids and movement that you can manage, within any personal restrictions. Ask for help early if you are losing weight, drinking less or finding everyday activity harder.

Keep checking the plan

A new medicine, fever, hospital admission, procedure or radiotherapy may change treatment timing. Tell the oncology team and take the wallet card or medicine list to other appointments.

Fertility, pregnancy and breastfeeding

Discuss these issues before the first dose whenever possible.

Fertility preservation

Ask whether treatment or the cancer may affect fertility and whether fertility preservation is appropriate before treatment. A change in periods or sexual function does not reliably establish infertility.

Written dates and methods

Record the agreed method, last treatment date and how long precautions continue. Different medicines have different intervals, so ask for clarification after a switch or combination change.

Sexual health

You can request a private discussion about intimacy, sexual symptoms or relationships. Ask for help if the agreed pregnancy precautions are difficult to follow.

Questions worth asking

Which tests are needed?

Blood count, kidney and liver function, glucose, thyroid tests and BRAF tumour status. Review autoimmune disease, previous immune reactions, transplantation, infection and all medicines. Confirm informed consent for this unlicensed melanoma dosing schedule and the local governance requirements. Blood count, kidney/liver tests, glucose and thyroid tests before every cycle, with symptom review during combination and maintenance phases. Cortisol and other organ tests are added when indicated. Recheck route, dose, interval and the maintenance start date when phases change. NCCP specifies ongoing monitoring for at least five months after the last dose. Immune effects can require longer follow-up, steroid tapering or lasting hormone replacement. Response scans and any further melanoma treatment follow the individual plan.

Can I use this without a phone?

Yes. The complete handbook, three-page summary and cut-out wallet card are printable. Ask the team to enter actual contact numbers and provide an updated paper plan when anything changes.

Try explaining the plan back: Can you explain your treatment setting, medicine schedule, next tests, the symptoms that need urgent advice and the number you would call?

The treatment this guide covers NCCP identifies nivolumab 3 mg/kg plus ipilimumab 1 mg/kg for melanoma as an unlicensed dosing posology in Ireland, despite reimbursement. Your clinician should explain this, the available alternatives and uncertainties, document informed consent under hospital policy, and follow the hospital Drug and Therapeutics Committee or equivalent acknowledgement process. CheckMate 511 primarily compared safety; it did not prove that this schedule has equal survival benefit to nivolumab 1 mg/kg plus ipilimumab 3 mg/kg.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Melanoma · combination induction · up to four doses

Active medicines: Nivolumab + Ipilimumab

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg orally once daily	Day 1, if prescribed under the low-risk plan Current NCIS V8 Table 1 p4. Review QT, electrolytes, constipation, hepatic dosing and interactions; this reference requires an individual prescription.
Metoclopramide	10 mg orally up to three times daily as needed	Subsequent days; duration to be prescribed Review contraindications, renal function, interactions and extrapyramidal adverse effects.

Additional phase-specific support

- The medicine list is a source-based low-risk reference for an individual consultant-approved/local prescription; it is separate from infusion-reaction premedication and immune-toxicity steroid treatment.
- NCCP classifies the combination phase as low emetogenic risk and nivolumab alone as minimal risk. Anti-sickness treatment is individual. Routine infusion premedication is not usually required; a previous reaction may alter that plan. Steroid or hormone treatment for immune effects must have a written follow-up plan.
- Blood count, kidney/liver tests, glucose and thyroid tests before every cycle, with symptom review during combination and maintenance phases. Cortisol and other organ tests are added when indicated. Recheck route, dose, interval and the maintenance start date when phases change.
- Immunotherapy can inflame healthy organs during treatment or after it stops. Contact the oncology team promptly for new or worsening diarrhoea, blood or mucus in stools, abdominal pain, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell every treating clinician that you are receiving or have received immunotherapy.
- Use effective contraception during treatment and for at least five months after the final nivolumab dose. Discuss fertility and breastfeeding before starting and tell the team promptly if pregnancy is possible. Keep the last-dose date on the treatment record.
- NCCP identifies nivolumab 3 mg/kg plus ipilimumab 1 mg/kg for melanoma as an unlicensed dosing posology in Ireland, despite reimbursement. Your clinician should explain this, the available alternatives and uncertainties, document informed consent under hospital policy, and follow the hospital Drug and

Therapeutics Committee or equivalent acknowledgement process. CheckMate 511 primarily compared safety; it did not prove that this schedule has equal survival benefit to nivolumab 1 mg/kg plus ipilimumab 3 mg/kg.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Melanoma · nivolumab maintenance

Active medicines: Nivolumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Nivolumab maintenance has minimal emetogenic risk; no automatic low-risk induction antiemetic prescription carries over. Routine infusion premedication is not usually required; prior reactions and local policy govern individual support.
- Blood count, kidney/liver tests, glucose and thyroid tests before every cycle, with symptom review during combination and maintenance phases. Cortisol and other organ tests are added when indicated. Recheck route, dose, interval and the maintenance start date when phases change.
- Immunotherapy can inflame healthy organs during treatment or after it stops. Contact the oncology team promptly for new or worsening diarrhoea, blood or mucus in stools, abdominal pain, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell every treating clinician that you are receiving or have received immunotherapy.
- Use effective contraception during treatment and for at least five months after the final nivolumab dose. Discuss fertility and breastfeeding before starting and tell the team promptly if pregnancy is possible. Keep the last-dose date on the treatment record.
- NCCP identifies nivolumab 3 mg/kg plus ipilimumab 1 mg/kg for melanoma as an unlicensed dosing posology in Ireland, despite reimbursement. Your clinician should explain this, the available alternatives and uncertainties, document informed consent under hospital policy, and follow the hospital Drug and Therapeutics Committee or equivalent acknowledgement process. CheckMate 511 primarily compared safety; it did not prove that this schedule has equal survival benefit to nivolumab 1 mg/kg plus ipilimumab 3 mg/kg.
- If a dose is delayed or held, obtain a revised written plan. The first two-weekly maintenance dose starts three weeks after the last combination dose; the first four-weekly dose starts six weeks afterwards. Do not calculate a different start from the maintenance interval alone. Earlier immune toxicity and permanent-stop decisions still apply.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Head / hormones

Immunotherapy

Severe headache, dizziness, profound tiredness, thirst or altered vision can signal pituitary, adrenal, thyroid or glucose problems. Symptoms may need urgent hormone tests and treatment.

3 Heart

Immunotherapy

Rare heart inflammation can cause chest pain, palpitations, faintness or breathlessness. Severe chest pain, collapse or major breathing difficulty is an emergency.

5 Liver

Immunotherapy

Liver inflammation may first appear in blood tests. Yellow eyes or skin, dark urine, severe nausea or right-sided abdominal pain also need prompt review.

7 Nerves / muscles

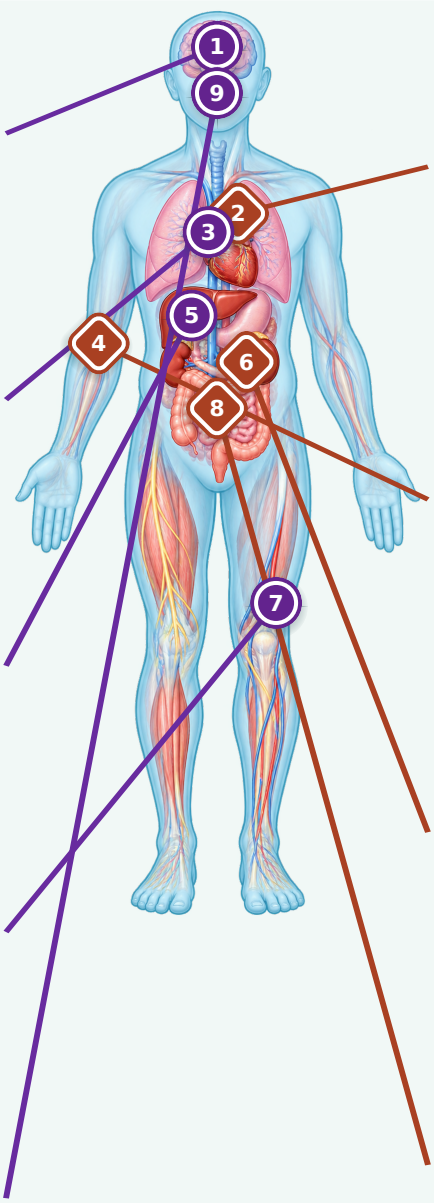
Immunotherapy

New weakness, drooping eyelids, double vision, numbness or difficulty walking, swallowing or breathing may be a rare nerve or muscle effect. Seek urgent help; do not wait for the next visit.

9 Eyes

Immunotherapy

Eye pain, redness, light sensitivity or changed vision can indicate inflammation. Get prompt assessment; do not assume it is ordinary tiredness.



2 Lungs

Immunotherapy

New cough, shortness of breath or chest discomfort can mean lung inflammation or infection. Report changes promptly, even with normal recent blood results.

4 Skin

Immunotherapy

Rash and itching can occur. Rapid spreading, blistering, peeling or sores involving the mouth, eyes or genitals require urgent assessment.

6 Kidneys / urine

Immunotherapy

Kidney inflammation may be detected in blood tests. Report reduced urine, blood in urine, swelling or a marked change in thirst.

8 Bowels

Immunotherapy

New diarrhoea, blood or mucus, severe cramps or abdominal pain can mean bowel inflammation. Record changes from your usual pattern and call early; severe pain or bleeding needs urgent assessment.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Immunotherapy can inflame healthy organs during treatment or after it stops. Contact the oncology team promptly for new or worsening diarrhoea, blood or mucus in stools, abdominal pain, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell every treating clinician that you are receiving or have received immunotherapy.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00551 v7a supportive-care section; current product information for medicine-specific precautions**
   https://healthservice.hse.ie/documents/6385/551_Nivolumab_3mgkg_Ipilimumab_1mgkg.pdf
2. **Lebbé et al. CheckMate 511, Journal of Clinical Oncology 2019**
   https://pubmed.ncbi.nlm.nih.gov/30811280/
3. **NCI: fatigue during cancer treatment**
   https://www.cancer.gov/about-cancer/treatment/side-effects/fatigue
4. **NCI: melanoma — patient information**
   https://www.cancer.gov/types/skin/patient/melanoma-treatment-pdq
5. **NCI: melanoma treatment — clinical evidence**
   https://www.cancer.gov/types/skin/hp/melanoma-treatment-pdq
6. **NCI: eating and nutrition during cancer treatment**
   https://www.cancer.gov/about-cancer/treatment/side-effects/nutrition
7. **NCI: palliative care during cancer treatment**
   https://www.cancer.gov/about-cancer/advanced-cancer/care-choices/palliative-care-fact-sheet
8. **NCCP NCIS medical-oncology antiemetics V8, Table 1 p4, low risk**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf